

NAME OF THE MEDICINE

GANFORT® PF Eye Drops

PRODUCT DESCRIPTION

GANFORT® PF is a sterile ophthalmic solution in a single-dose container.

Each mL of GANFORT® PF contains:

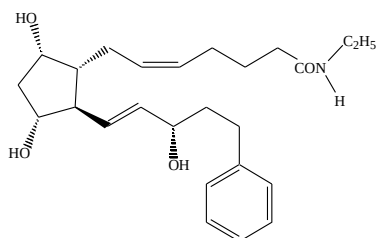
ACTIVES: Bimatoprost 0.3 mg/mL and timolol 5.0 mg/mL

EXCIPIENTS: Sodium chloride, sodium phosphate dibasic heptahydrate, citric acid monohydrate; and purified water. Hydrochloric acid or sodium hydroxide may be added to adjust pH.

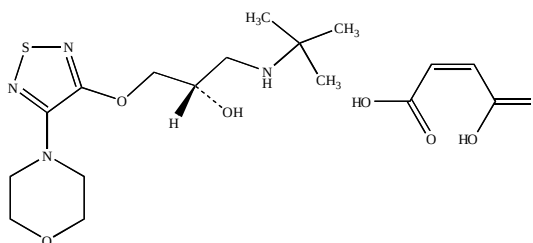
Physical Appearance:

GANFORT® PF eye drops is a clear and colourless solution.

Chemical structure:



(structure of bimatoprost)



(structure of timolol maleate)

CAS Registry No. 155206-00-1

CAS Registry No. 26921-17-5

Bimatoprost is a synthetic prostamide analogue for ophthalmic use. It is a white to off-white powder and is very soluble in ethyl alcohol, methyl alcohol and slightly soluble in water.

Timolol maleate is a non-selective beta-adrenergic receptor blocking agent. It is a white, odourless, crystalline powder which is soluble in water, methanol and alcohol.

INDICATIONS

GANFORT® PF eye drops are indicated for the reduction of intraocular pressure (IOP) in patients with open-angle glaucoma or ocular hypertension who are insufficiently responsive to topical beta-blockers or prostaglandin analogues.

DOSAGE AND ADMINISTRATION

Posology

Recommended dosage in adults (including older people)

The recommended dose is one drop of GANFORT® PF in the affected eye(s) once daily, administered either in the morning or in the evening. It should be administered at the same time each day.

Existing literature data for GANFORT® (multi-dose formulation) suggest that evening dosing may be more effective in IOP lowering than morning dosing. However, consideration should be given to the likelihood of compliance when considering either morning or evening dosing.

The single-dose container is for single use only; one container is sufficient to treat both eyes. Any unused solution should be discarded immediately after use. If one dose is missed, treatment should continue with the next dose as planned. The dose should not exceed one drop in the affected eye(s) daily.

Renal and hepatic impairment

GANFORT® PF has not been studied in patients with hepatic or renal impairment. Therefore caution should be used in treating such patients.

Paediatric population

The safety and efficacy of GANFORT® PF in children aged less than 18 years has not been established. No data are available.

Method of administration

If more than one topical ophthalmic medicinal product is to be used, each one should be instilled at least 5 minutes apart.

When using nasolacrimal occlusion or closing the eyelids for 2 minutes, the systemic absorption is reduced. This may result in a decrease in systemic side effects and an increase in local activity

PHARMACOLOGY

Pharmacotherapeutic group: Ophthalmologicals; beta-blocking agents; ATC code: S01ED51

Pharmacodynamics

Mechanism of action

GANFORT® PF consists of two active substances: bimatoprost and timolol maleate. These two components decrease elevated intraocular pressure (IOP) by complementary mechanisms of action and the combined effect results in additional IOP reduction compared to either compound administered alone. GANFORT® PF has a rapid onset of action.

Bimatoprost

Bimatoprost is a potent ocular hypotensive active substance. It is a synthetic prostamide, structurally related to prostaglandin F_{2α} (PGF_{2α}) that does not act through any known prostaglandin receptors. Bimatoprost selectively mimics the effects of newly discovered biosynthesised substances called prostamides. The prostamide receptor, however, has not yet been structurally identified. The mechanism of action by which bimatoprost reduces intraocular pressure in man is by increasing aqueous humour outflow through the trabecular meshwork and enhancing uveoscleral outflow.

Timolol

Timolol is a beta₁ and beta₂ non-selective adrenergic receptor blocking agent that does not have significant intrinsic sympathomimetic, direct myocardial depressant, or local anaesthetic (membrane-stabilising) activity. Timolol lowers IOP by reducing aqueous humour formation. The precise mechanism of action is not clearly established, but inhibition of the increased cyclic AMP synthesis caused by endogenous beta-adrenergic stimulation is probable.

Clinical effects

A 12-week (double-masked, randomized, parallel group) clinical study compared the efficacy and safety of GANFORT® PF with GANFORT® (multi-dose formulation) in patients with glaucoma or ocular hypertension. GANFORT® PF achieved noninferior IOP-lowering efficacy to GANFORT® (multi-dose formulation): the upper limit of the 95% CI of the between-treatment difference was within the pre-defined 1.5 mm Hg margin at each timepoint evaluated (hours 0, 2, and 8) at week 12 (for the primary

analysis), and also at weeks 2 and 6, for mean worse eye IOP change from baseline (worse eye IOP refers to the eye with the higher mean diurnal IOP at baseline). In fact, the upper limit of the 95% CI did not exceed 0.14 mm Hg at week 12.

Both treatment groups showed statistically and clinically significant mean decreases from baseline in worse eye IOP at all follow up timepoints throughout the study ($p < 0.001$). Mean changes from baseline worse eye IOP ranged from -9.16 to -7.98 mm Hg for GANFORT® PF group, and from -9.03 to -7.72 mm Hg for the GANFORT® (multi-dose formulation) group across the 12-week study.

GANFORT® PF also achieved equivalent IOP-lowering efficacy to GANFORT® (multi-dose formulation) in average eye and worse eye IOP at each follow-up timepoint at weeks 2, 6 and 12.

Based on studies of GANFORT® (multi-dose formulation), the IOP-lowering effect of GANFORT® is non-inferior to that achieved by adjunctive therapy of bimatoprost (once daily) and timolol (twice daily). Existing literature data for GANFORT® (multi-dose formulation) suggest that evening dosing may be more effective in IOP lowering than morning dosing. However, consideration should be given to the likelihood of compliance when considering either morning or evening dosing.

Paediatric population

The safety and efficacy of GANFORT® PF in children aged less than 18 years has not been established.

Pharmacokinetics

GANFORT® medicinal product

Plasma bimatoprost and timolol concentrations were determined in a crossover study comparing the monotherapy treatments to GANFORT® (multi-dose formulation) treatment in healthy subjects. Systemic absorption of the individual components was minimal and not affected by co-administration in a single formulation.

In two 12-month studies of GANFORT® (multi-dose formulation) in which systemic absorption was measured, no accumulation was observed of either of the individual components.

Bimatoprost

Bimatoprost penetrates the human cornea and sclera *in vitro*.

After ocular administration, the systemic exposure of bimatoprost is very low with no accumulation over time. After once daily ocular administration of one drop of 0.03% bimatoprost to both eyes for two weeks, blood concentrations peaked within 10 minutes after dosing and declined to below the lower limit of detection (0.025 ng/mL) within 1.5 hours after dosing. Mean C_{max} and AUC_{0-24hrs} values were similar on days 7 and 14 at approximately 0.08 ng/mL and 0.09 ng•hr/mL respectively, indicating that a steady drug concentration was reached during the first week of ocular dosing.

Bimatoprost is moderately distributed into body tissues and the systemic volume of distribution in humans at steady-state was 0.67 l/kg. In human blood, bimatoprost resides mainly in the plasma. The plasma protein binding of bimatoprost is approximately 88%.

Bimatoprost is the major circulating species in the blood once it reaches the systemic circulation following ocular dosing in humans. Bimatoprost then undergoes oxidation, N-deethylation and glucuronidation to form a diverse variety of metabolites.

Bimatoprost is eliminated primarily by renal excretion. Up to 67% of an intravenous dose of radiolabelled bimatoprost administered to healthy volunteers was excreted in the urine, 25% of the dose was excreted via the faeces. The elimination half-life, determined after intravenous administration, was approximately 45 minutes, the total blood clearance of unchanged bimatoprost was 1.5 L/hr/kg.

Characteristics in older people

After twice daily dosing of bimatoprost 0.3 mg/ml, the mean AUC_{0-24hr} value of 0.0634 ng•hr/mL bimatoprost in the elderly (subjects 65 years or older) was statistically significantly higher than that of 0.0218 ng•hr/mL in young healthy adults, suggesting the existence of an age effect. However, this finding is not clinically relevant as systemic exposure for both elderly and young subjects remained very low from ocular dosing. There was no accumulation of bimatoprost in the blood over time and the safety profile was similar in elderly and young patients.

Timolol

After ocular administration of a 0.5% eye drops solution in humans undergoing cataract surgery, peak timolol concentration was 898 ng/ml in the aqueous humour at one hour post-dose. Part of the dose is absorbed systemically where it is extensively metabolised in the liver. The half-life of timolol in plasma is about 4 to 6 hours. Timolol is partially metabolised by the liver with timolol and its metabolites excreted by the kidney. Timolol is not extensively bound to plasma.

CONTRAINDICATIONS

- Hypersensitivity to the active substances or to any of the excipients.
- Reactive airway disease including bronchial asthma or a history of bronchial asthma, severe chronic obstructive pulmonary disease.
- Sinus bradycardia, sick sinus syndrome, sino-atrial nodal block, second or third degree atrioventricular block not controlled with a pacemaker; overt cardiac failure, cardiogenic shock.

WARNING AND PRECAUTIONS

General:

Like other topically applied ophthalmic agents, GANFORT® PF may be absorbed systemically. No enhancement of the systemic absorption of the individual active substances has been observed with GANFORT® (multi-dose formulation). Due to the beta-adrenergic component, timolol, the same types of cardiovascular and pulmonary adverse reactions as seen with systemic beta-blockers may occur.

Cardiac disorders: Patients with cardiovascular diseases (e.g. coronary heart disease, Prinzmetal's angina and cardiac failure) and receiving hypotension therapy with beta-blockers should be critically assessed and therapy with other active substances should be considered. Patients with cardiovascular diseases should be watched for signs of deterioration of these diseases and of adverse reactions.

Due to the negative effect on conduction time, beta-blockers should only be given with caution to patients with first degree heart block.

Respiratory Disorder:

Respiratory reactions, including death due to bronchospasm in patients with asthma, have been reported following administration of some ophthalmic beta-blockers.

GANFORT® PF should be used with caution in patients with mild/moderate chronic obstructive pulmonary disease (COPD) and only if the potential benefit outweighs the potential risk..

Endocrine disorders:

Beta-adrenergic blocking medicinal products should be administered with caution in patients subject to spontaneous hypoglycaemia or in patients with labile diabetes as beta-blockers may mask the signs and symptoms of acute hypoglycemia.

Beta-blockers may also mask the signs of hyperthyroidism.

Vascular disorders: Patients with severe peripheral circulatory disturbance/disorders (i.e. severe forms of Raynaud's disease or Raynaud's syndrome) should be treated with caution.

Other beta-blocking agents: The effect on intra-ocular pressure or the known effects of systemic beta-blockade may be potentiated when timolol is given to patients already receiving a systemic beta-blocking agent. The response of these patients should be closely observed. The use of two topical beta-adrenergic blocking agents is not recommended (see section *Interactions With Other Medicaments*)

Surgical anesthesia: Beta-blocking ophthalmological preparations may block systemic beta-agonist effects e.g. of adrenaline. The anaesthesiologist should be informed when the patient is receiving timolol.

Hepatic: In patients with a history of mild liver disease or abnormal alanine aminotransferase (ALT), aspartate aminotransferase (AST) and/or bilirubin at baseline, bimatoprost eye drops had no adverse reactions on liver function over 24 months. There are no known adverse reactions of ocular timolol on liver function.

Corneal diseases: Ophthalmic beta-blockers may induce dryness of eyes. Patients with corneal diseases should be treated with caution

Choroidal detachment: Choroidal detachment has been reported with administration of aqueous suppressant therapy (e.g. timolol, acetazolamide) after filtration procedures.

Anaphylaxis reactions: While taking beta-blockers, patients with a history of atopy or a history of severe anaphylactic reaction to a variety of allergens may be more reactive to repeated challenge with such allergens. Such patients may be unresponsive to the usual dose of epinephrine used to treat anaphylactic reactions.

Ocular:

Before treatment is initiated, patients should be informed of the possibility of eyelash growth, and periorbital skin hyperpigmentation since these have been observed during treatment with GANFORT® PF. Increased brown iris pigmentation has also been observed during treatment with GANFORT® (multi-dose formulation). Increased iris pigmentation is likely to be permanent, and may lead to differences in appearance between the eyes if only one eye is treated. After discontinuation of GANFORT®, pigmentation of iris may be permanent. After 12 months of treatment with GANFORT® (multi-dose formulation), the incidence of iris pigmentation was 0.2%. After 12 months of treatment with bimatoprost eye drops alone, the incidence was 1.5% and did not increase following 3 years of treatment. The pigmentation change is due to increased melanin content in the melanocytes rather than to an increase in the number of melanocytes. The long term effects of increased iridial pigmentation are not known. Iris color changes seen with ophthalmic administration of bimatoprost may not be noticeable for several months to years. Neither nevi nor freckles of the iris appear to be affected by treatment. Periorbital tissue pigmentation has been reported to be reversible in some patients.

Macular oedema, including cystoid macular oedema has been reported with GANFORT® (multi-dose formulation). Therefore, GANFORT® PF should be used with caution in aphakic patients, in pseudophakic patients with a torn posterior lens capsule, or in patients with known risk factors for macular oedema (e.g. intraocular surgery, retinal vein occlusions, ocular inflammatory disease and diabetic retinopathy).

GANFORT® should be used with caution in patients with active intraocular inflammation (e.g. uveitis) because the inflammation may be exacerbated.

Skin: There is a potential for hair growth to occur in areas where GANFORT® solution comes repeatedly in contact with the skin surface. Thus, it is important to apply GANFORT® as instructed and avoid it running onto the cheek or other skin areas.

Other conditions: GANFORT® PF has not been studied in patients with inflammatory ocular conditions, neovascular, inflammatory, angle-closure, congenital or narrow-angle glaucoma.

In studies of bimatoprost 0.3 mg/mL in patients with glaucoma or ocular hypertension, it has been shown that more frequent exposure of the eye to more than 1 dose of bimatoprost daily may decrease the

IOP-lowering effect. Patients using GANFORT® with other prostaglandin analogues should be monitored for changes to their intraocular pressure.

Preclinical Safety Data

Repeated dose ocular toxicity studies on GANFORT® (multi-dose formulation) showed no special additional hazard for humans. The ocular and systemic safety profile of the individual components is well established.

Bimatoprost: Non-clinical data reveal no special hazard for humans based on conventional studies of safety pharmacology, genotoxicity, carcinogenic potential. Studies in rodents produced species-specific abortion at systemic exposure levels 33- to 97-times that achieved in humans after ocular administration.

Monkeys administered ocular bimatoprost concentrations of $\geq 0.03\%$ daily for 1 year had an increase in iris pigmentation and reversible dose-related periocular effects characterised by a prominent upper and/or lower sulcus and widening of the palpebral fissure. The increased iris pigmentation appears to be caused by increased stimulation of melanin production in melanocytes and not by an increase in melanocyte number. No functional or microscopic changes related to the periocular effects have been observed, and the mechanism of action for the periocular changes is unknown.

Timolol: Non-clinical data reveal no special hazard for humans based on conventional studies of safety pharmacology, repeated dose toxicity, genotoxicity, carcinogenic potential, toxicity to reproduction.

PREGNANCY and LACTATION

Pregnancy:

There are no adequate data from the use of the bimatoprost / timolol fixed combination in pregnant women. GANFORT® PF should not be used during pregnancy unless clearly necessary.

Bimatoprost

No adequate clinical data in exposed pregnancies are available. Animal studies have shown reproductive toxicity at high maternotoxic doses (see section *Preclinical safety data*)

Timolol

Epidemiological studies have not revealed malformative effects but have shown a risk for intra uterine growth retardation when beta-blockers are administered by the oral route. In addition, signs and symptoms of beta-blockade (e.g. bradycardia, hypotension, respiratory distress and hypoglycaemia) have been observed in the neonate when beta-blockers have been administered until delivery. If GANFORT® PF is administered until delivery, the neonate should be carefully monitored during the first days of life. Animal studies with timolol have shown reproductive toxicity at doses significantly higher than would be used in clinical practice (see section *Preclinical safety data*).

Breast-feeding:

Timolol

Beta-blockers are excreted in breast milk. However, at therapeutic doses of timolol in eye drops it is not likely that sufficient amounts would be present in breast milk to produce clinical symptoms of beta-blockade in the infant. To reduce the systemic absorption, see section 4.2.

Bimatoprost

It is not known if bimatoprost is excreted in human breast milk but it is excreted in the milk of the lactating rat. GANFORT® PF should not be used by breast-feeding women.

Fertility

There are no data on the effects of GANFORT® PF on human fertility.

INTERACTIONS WITH OTHER MEDICINES

Specific drug interaction studies have not been conducted with GANFORT® PF eye drops.

Patients who are receiving a systemic (e.g., oral or intravenous) beta-adrenergic blocking agent and GANFORT® PF should be observed for potential additive effects of beta-blockade, both systemic and on intraocular pressure.

There is a potential for additive effects resulting in hypotension, and/or marked bradycardia when eye drops containing timolol are administered concomitantly with oral calcium channel blockers, guanethidine, or beta-blocking agents, anti-arrhythmics (including amiodarone), digitalis glycosides or parasympathomimetics, and other anti-hypertensives.

Although timolol has little or no effect on the size of the pupil, mydriasis has occasionally been reported when timolol has been used with mydriatic agents such as adrenaline.

The hypertensive reaction to sudden withdrawal of clonidine can be potentiated when taking beta-blockers.

Potentiated systemic beta-blockade (e.g. decreased heart rate, depression) has been reported during combined treatment with CYP2D6 inhibitors (e.g., quinidine, selective serotonin reuptake inhibitors (SSRIs), and timolol).

Beta-blockers may increase the hypoglycaemic effect of antidiabetic agents. Beta-blockers can mask the signs and symptoms of hypoglycaemia.

Effects on ability to drive and use machines:

GANFORT® PF has negligible influence on the ability to drive and use machines. As with any ocular treatment, if transient blurred vision occurs at instillation, the patient should wait until the vision clears before driving or using machinery.

SIDE EFFECTS

GANFORT® PF

Summary of the safety profile

The adverse reactions reported in the clinical study using GANFORT® PF were limited to those earlier reported for either GANFORT® (multi-dose formulation) or for the single active substances bimatoprost or timolol. No new adverse reactions specific for GANFORT® PF have been observed in clinical studies.

The majority of adverse reactions reported with GANFORT® PF were ocular, mild in severity and none were serious. Based on a 12-week study of GANFORT® PF administered once daily, the most commonly reported adverse reaction with GANFORT® PF was conjunctival hyperaemia (mostly trace to mild and thought to be of a non-inflammatory nature) in approximately 21% of patients and led to discontinuation in 1.4% of patients.

Tabulated list of adverse reactions

Table 1 presents the adverse reactions that were reported during clinical studies of both GANFORT® PF and GANFORT® (multi-dose formulations) (within each frequency grouping, adverse reactions are presented in order of decreasing seriousness) or in the post-marketing period.

The frequency of possible adverse reactions listed below is defined using the following convention:

Very common	≥1/10
Common	≥1/100 to <1/10
Uncommon	≥1/1,000 to <1/100
Rare	≥1/10,000 to <1/1,000
Very rare	<1/10,000
Not known	Frequency cannot be estimated from available data

Table 1

System Organ Class	Frequency	Adverse reaction
<i>Immune system disorders</i>	Not known	hypersensitivity reactions including signs or symptoms of allergic dermatitis, angioedema, eye allergy
<i>Psychiatric disorders</i>	Not known	Insomnia ² , nightmare ²
<i>Nervous system disorders</i>	Common	headache, dizziness ²
	Not known	Dysgeusia ²
<i>Eye disorders</i>	Very common	conjunctival hyperaemia.
	Common	punctate keratitis, corneal erosion ² , burning sensation ² , conjunctival irritation ¹ , eye pruritus, stinging sensation in the eye ² , foreign body sensation, dry eye, erythema of eyelid, eye pain, photophobia, eye discharge ² , visual disturbance ² , eyelid pruritus, visual acuity worsened ² , blepharitis ² , eyelid oedema, eye irritation, lacrimation increased, growth of eyelashes.
	Uncommon	iritis ² , conjunctival oedema ² , eyelid pain ² , abnormal sensation in the eye ¹ , asthenopia, trichiasis ² , iris hyperpigmentation ² , deepening of eyelid sulcus ² , eyelid retraction ² , eyelash discolouration (darkening) ¹ .
	Not known	cystoid macular oedema ² , eye swelling, vision blurred ² , Ocular discomfort
<i>Cardiac disorders</i>	Not known	Bradycardia
<i>Respiratory, thoracic and mediastinal disorders</i>	Common	Rhinitis ²

	Uncommon	dyspnoea
	Not known	bronchospasm (predominantly in patients with pre-existing bronchospastic disease) ² , asthma
<i>Skin and subcutaneous tissue disorders</i>	Common	blepharal pigmentation ² , hirsutism ² , skin hyperpigmentation (periocular).
	Not known	Alopecia, Skin discoloration (periocular)
<i>Vascular disorders</i>	Not known	Hypertension
<i>General disorders and administration site conditions</i>	Not known	fatigue

¹adverse reactions only observed with GANFORT® PF

²adverse reactions only observed with GANFORT® (multi-dose formulation)

Like other topically applied ophthalmic drugs, GANFORT (bimatoprost/timolol) is absorbed into the systemic circulation. Absorption of timolol may cause similar undesirable effects as seen with systemic beta-blocking agents. The incidence of systemic ADRs after topical ophthalmic administration is lower than for systemic administration.

Additional adverse reactions that have been seen with either of the active substances (bimatoprost or timolol), and may potentially occur also with GANFORT® are listed below in Table 2:

Table 2

System Organ Class	Adverse reaction
<i>Immune system disorders</i>	systemic allergic reactions including anaphylaxis ¹
<i>Metabolism and nutrition disorders</i>	hypoglycaemia ¹
<i>Psychiatric disorders</i>	depression ¹ , memory loss ¹
<i>Nervous system disorders</i>	syncope ¹ , cerebrovascular accident ¹ , increase in signs and symptoms of myasthenia gravis ¹ , paraesthesia ¹ , cerebral ischaemia ¹
<i>Eye disorders</i>	decreased corneal sensitivity ¹ , diplopia ¹ , ptosis ¹ , choroidal detachment following filtration surgery (see section 4.4) ¹ , keratitis ¹ , blepharospasm ² , retinal haemorrhage ² , uveitis ² , eye discharge ² , periorbital and lid changes associated with periorbital fat atrophy and skin tightness resulting in deepening of eyelid sulcus, eyelid ptosis, enophthalmos and eyelid retraction.
<i>Cardiac disorder</i>	atrioventricular block ¹ , cardiac arrest ¹ , arrhythmia ¹ , cardiac failure ¹ , congestive heart failure ¹ , chest pain ¹ , palpitations ¹ , oedema ¹
<i>Vascular disorders</i>	hypotension ¹ , Raynaud's phenomenon ¹ , cold hands and feet ¹
<i>Respiratory, thoracic and mediastinal disorders</i>	Asthma exacerbation ² , COPD exacerbation ² , cough ¹
<i>Gastrointestinal disorders</i>	nausea ^{1,2} , diarrhoea ¹ , dyspepsia ¹ , dry mouth ¹ , abdominal pain ¹ , vomiting ¹

<i>Skin and subcutaneous tissue disorders</i>	psoriasiform rash ¹ or exacerbation of psoriasis ¹ , skin rash ¹
<i>Musculoskeletal and connective tissue disorders</i>	myalgia ¹
<i>Reproductive system and breast disorders</i>	sexual dysfunction ¹ , decreased libido ¹
<i>General disorders and administration site conditions</i>	asthenia ^{1,2}
<i>Investigations</i>	liver function tests (LFT) abnormal ²

¹ adverse reactions observed with Timolol monotherapy

² adverse reactions observed with Bimatoprost monotherapy

Adverse reactions reported in phosphate containing eye drops

Cases of corneal calcification have been reported very rarely in association with the use of phosphate containing eye drops in some patients with significantly damaged corneas.

SYMPTOMS AND TREATMENT OF OVERDOSE

A topical overdose with GANFORT® PF is not likely to occur or to be associated with toxicity.

Bimatoprost

If GANFORT® PF is accidentally ingested, the following information may be useful: in 2-week oral mice and rats studies, doses of bimatoprost up to 100 mg/kg/day did not produce any toxicity; this corresponds to a human equivalent dose of 8.1 and 16.2 mg/kg, respectively. These doses are at least 7.5 times higher than the amount of bimatoprost in an accidental dose of the entire contents of a carton of GANFORT® PF (90 single-dose containers x 0.4 mL; 36 mL) in a 10 kg child [(36 mL*0.3 mg/mL bimatoprost)/10 kg; 1.08 mg/kg].

Timolol

Symptoms of systemic timolol overdose include: bradycardia, hypotension, bronchospasm, headache, dizziness, shortness of breath, and cardiac arrest. A study of patients with renal failure showed that timolol did not dialyse readily.

If overdose occurs treatment should be symptomatic and supportive.

PRODUCT PRESENTATION

GANFORT® PF (bimatoprost) 0.3 mg/mL and (timolol as maleate) 5.0 mg/mL eye drops sterile solution is supplied in single-dose low density polyethylene (LDPE) containers with a twist-off tab. Each single-dose contains 0.4 mL solution.

The two pack sizes are:

- 5 x 0.4 mL single dose containers (physician samples). Five single-dose containers are connected in a strip and packaged in an aluminium pouch.
- 30 x 0.4mL single dose containers. Each five single-dose containers are connected in a strip and two strips are packaged in an aluminium pouch. Three pouches are then placed in an outer carton.

Not all pack sizes may be marketed.

STORAGE CONDITION

Store below 30°C. Keep the single dose container in the aluminium pouch until use. Protect from light and moisture. Do not freeze. Once the pouch is opened, keep the single-dose ampoules in the pouch and place the pouch back in carton to protect from light and moisture, use within 10 days. Discard the opened single-dose container immediately after use.

SHELF LIFE

2 years.

Date of last revision: 5 March 2024 (CCDS v4.0 June 2021)

Manufactured by:

Allergan Pharmaceuticals Ireland

Castlebar Road, Westport, County Mayo, Ireland

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